

Session Objectives

- Discuss the history of the FPR technique
- Explain the principles of the FPR technique
- Explain the physiologic mechanisms involved
- Discuss the indications and contraindications of the FPR technique
- Explain how to apply the principles of FPR to treat any given somatic dysfunction based on your knowledge of anatomy and the function of the given region.

Basic Principles of FPR

What are the basics of performing the technique?

- Neutralize the position of the dysfunction
- Add a facilitating force to the point being treated.
(Facilitating force = Compress, traction, or torque)
- Maintain the facilitating force and guide the dysfunction into its freedom & stay there for 3-5 seconds
- Release facilitating force and guide dysfunction back to baseline position and reassess

Indications

- Any condition where somatic dysfunction is involved
 - Note: Treat larger muscle groups first before treating joints
 - ie. In someone with neck pain secondary to whiplash, treat the cervical soft tissue first, then move on to the spinal segments.

Contraindications of FPR (1 of 3)

- Patient unable to tolerate treatment positions
 - Modify technique or use a different technique
- Radicular pain when performing to c-spine using compression (similar to Spurling Maneuver)
 - Reposition, try with traction, or try another technique

Contraindications of FPR (2 of 3)

- Any condition of the region being treated where introduction of torque, traction, or compression is contraindicated
 - Hip replacement (treatment position for herniated disc pain)
 - Shoulder pathology (treatment position for Rib 1)
 - Chronic shoulder dislocation/separation (any treatment that moves and /or puts force into the shoulder)

Contraindications of FPR (3 of 3)

- Recent trauma, fracture or dislocation at area involved with treatment
- Caution (but, not contraindication)
 - Osteoporosis
 - Malignancy
 - Rheumatologic Disorders
 - Congenital malformations
 - Stenosis
 - Other clinical diseases (use your clinical judgement)

Case (continued)

PMHx: childhood asthma

PSx: no surgeries

Soc Hx: denies alcohol, drugs, tobacco use

Family History: both parents and 2 siblings alive and well

ROS: Only positive for the back pain

Case (continued)

Plan:

Low Back pain – Hamstring, psoas, QL, Paraspinal mm. stretches. Stretch each region for 20 seconds, relax the stretch for a few seconds, restretch 20 seconds and relax 2 more times.

Muscle spasm – Stretches as above

Somatic Dysfunction – Lumbar

OMT – Soft tissue, MET, FPR

Somatic Dysfunction – Lower Extremity

OMT - MET

Summary

FPR:

- Passive, indirect technique
- Physiologic mechanism - Muscle spindle
- Method - Neutralize, add facilitating force, guide to freedoms, return to baseline, reassess